

LRI Emergency Department

Occupational blood borne virus (BBV) exposure

Use to manage UHL healthcare workers (HCW) outside Occupational Health Department working hours (8:30–16:30 Mon-Fri) and other professional groups (e.g. police) after exposure incidents to HBV, HCV or HIV

Disclaimer: This is a clinical template; clinicians should always use judgment when managing individual patients

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Review due Jul25 . Version 53

Created by Martin Wiese (as Appendix H&I, UHL BBV Occupational Exposure Policy . Trust Ref B42/2007)

Patient details

Full name

DoB

Unit number

(use sticker if available)

① Mucocutaneous exposure?

☐ YES, as one of the below was splashed

Mucous membrane

Eye☐

Inside of the nose☐

Inside of mouth☐

Non-intact skin

Cut☐

Abrasions☐

Eczema☐

Other (please describe below)☐

☐ NO, as only intact skin was splashed

- * First aid measures
- Exposed area to be washed liberally with soap & water **but without scrubbing**
 - Encourage free bleeding of wounds **without sucking**
 - Copious irrigation of mucous membranes; conjunctivae both **before and after** removal of any contact lenses

DD/MM/YY

Date of exposure

HH:MM

Time of exposure

DD/MM/YY

Date of assessment

HH:MM

Time of assessment

HH:MM

Target time of 1st HIV-PEP dose

HH:MM

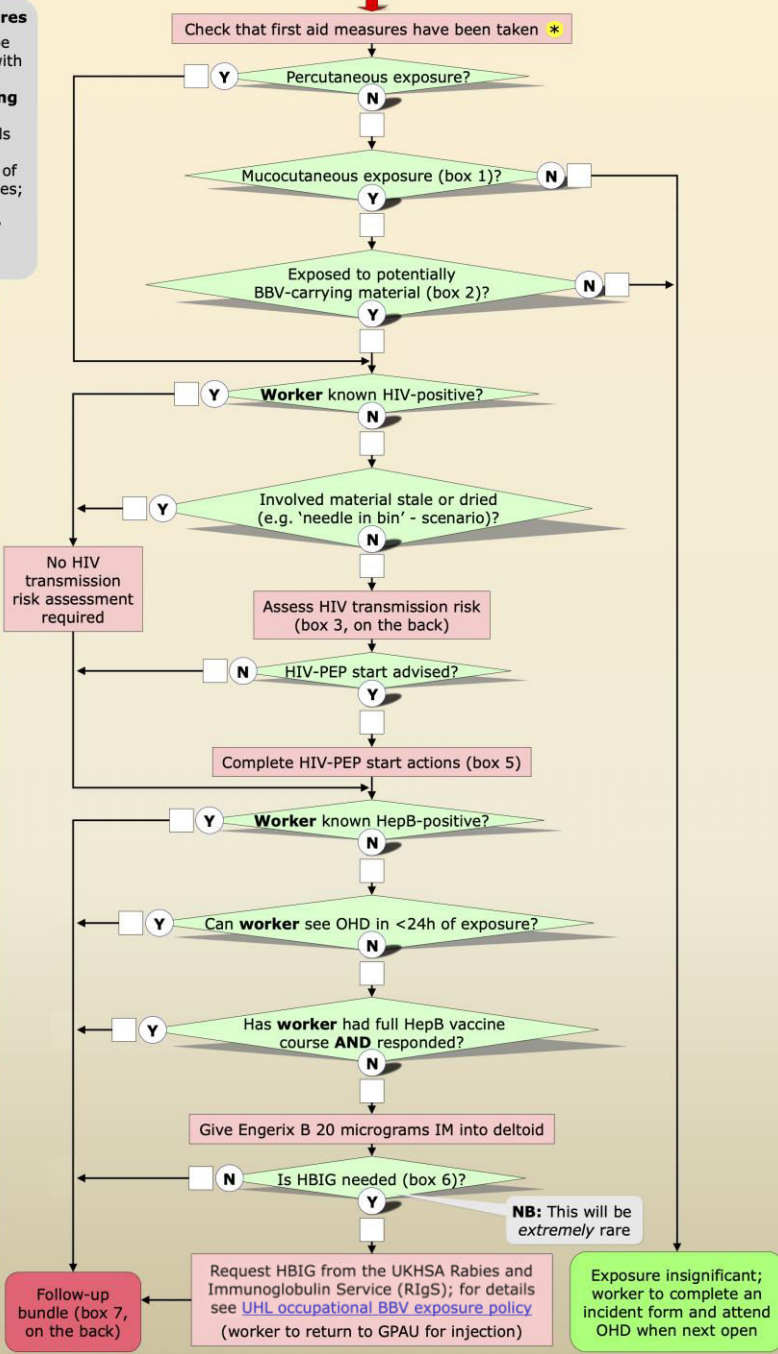
Actual time of 1st HIV-PEP dose

DD/MM/YY

Target date of HepB vaccine / HBIG

HH:MM

Target time of HepB vaccine / HBIG



② Could material carry BBV?

☐ YES, because one of the below involved

Amniotic fluid☐

Blood☐

Breast milk☐

Cerebrospinal fluid☐

Fluid from burns or skin lesions☐

Pericardial fluid☐

Peritoneal fluid☐

Pleural fluid☐

Semen☐

Synovial fluid☐

Unfixed tissue or organ☐

Vaginal secretions☐

Visibly bloodstained (or dentistry-related) saliva☐

Other visibly bloodstained body fluid (give details below)☐

☐ NO, as none of the above involved

④ Need for HIV-PEP cautions?

☐ YES, at least one of the below

Pregnancy (consider testing as applicable)☐

Known or clinically suspected eGFR<50☐

Source HIV+ **AND** on antiretroviral therapy☐

Affected worker is taking phenobarbital, rifampicin, carbamazepine, phenytoin or oxcarbazepine, or takes regular calcium, magnesium, iron or aluminium, which is often found in multi-vitamin supplements or indigestion remedies☐

Affected worker has already been taking PrEP (pre-exposure prophylaxis)☐

☐ NO, none of the above

⑤ HIV-PEP start actions

unless IDU consultant advised differently

Take an HIV PEP pack from Majors clean utility (TTO pre-pack cupboard)☐

Read 'Information for prescribers' sheet (stapled to the outside of the pack) and discuss all items stated on its second page with the patient☐

Prescribe PEP exactly as per instructions in the pack☐

Ensure first dose is taken ASAP☐

Record time dose taken in box on the left☐

Place prescription into labeled plastic wallet on the side of the TTO pre-pack cupboard☐

Record details in the 'Pre-pack Medicine Register (next to TTO pre-pack cupboard)☐

⑥ HB immunoglobulin needed?

☐ YES, as one of the two scenarios below

Affected worker has never been vaccinated against HepB **AND** source patient is known to be HepB-positive☐

OR

Affected worker is known non-responder to HepB vaccine **AND** source patient's HepB status is positive or unknown☐

☐ NO, as neither of the scenarios above

Manage any significant wounds as per standard practices and consider tetanus vaccination status

This patient was managed by

Print name

Signature

Role

③ HIV transmission risk assessment matrix

		Source HIV status details			
		Positive	Unknown	Unknown	Positive
tick applicable exposure & source HIV status details		☐ Viral load detectable or unknown, e.g. source not on treatment	☐ High prevalence group ☐ Man who has sex with men ☐ Injecting drug user ☐ Originating from a country with HIV prevalence ≥1% (see bit.ly/2Ln0gXf or Wikipedia page) Country Prevalence %	☐ Not from high prevalence group	☐ Viral load undetectable (source has confirmed that their viral load [VL] is <200 copies/mL)
		PEP not indicated and advice from the infectious diseases team not needed: Transmission risk of HIV less than 1 in 10.000			
Exposure details	☐ Percutaneous exposure ☐ Used hollow-bore needle ☐ Used razor ☐ Bone fragment (e.g. when inserting a chest drain) ☐ Other sharp object; details: e.g. used scalpel blade, suture needle or trochar	Start PEP NB: If there is a need for caution (see front, box 4), seek advice from on-call IDU consultant first	Seek advice from on-call IDU consultant		
	☐ Mucocutaneous exposure to blood or other virus-containing fluid				
	☐ Bite with ALL THREE of the conditions below ☐ Source has untreated HIV infection AND ☐ Some of biter's blood was present in their mouth AND ☐ Bite has punctured the skin	Seek advice from on-call IDU consultant			
	☐ Bite without the conditions above				

⑦ Follow-up bundle

Advise affected worker to

- ☐ Call the Occupational Health Department (OHD - if not already done) before leaving the ED to record a brief message with a personal phone number on the OHD voicemail **0116 258 5307**

Visit the Occupational Health Department (OHD) as directed when called back by the OHD
 - ☐ Non-UHL staff who are sure that their organisation has its own OHD and who do not require HIV-PEP attend their own OHD
 - ☐ Non-UHL staff who are unsure where to go or have been started on HIV-PEP attend LRI OHD
 - ☐ Record incident on DATIX
 - ☐ Return to working as per usual (unless, rarely, more severe physical injury precludes this)
 - ☐ Practice safer sex (i.e. use condom) and not donate blood or other body fluids / tissue pending further advice from OHD
 - ☒ **ONLY If HBIG needed**, return to GPAU to receive HBIG injection, as arranged by ED clinician
 - ☐ Ask their own team to arrange source patient testing for HBV, HCV and HIV unless recent results are available
- Please consider printing the following resources that the affected worker can take back to their team to aid source testing
- [Source patient team guidance notes](#) - Occupational BBV exposure policy, **Appendix J**
 - [Source patient information leaflet](#) - Occupational BBV exposure policy, **Appendix K**